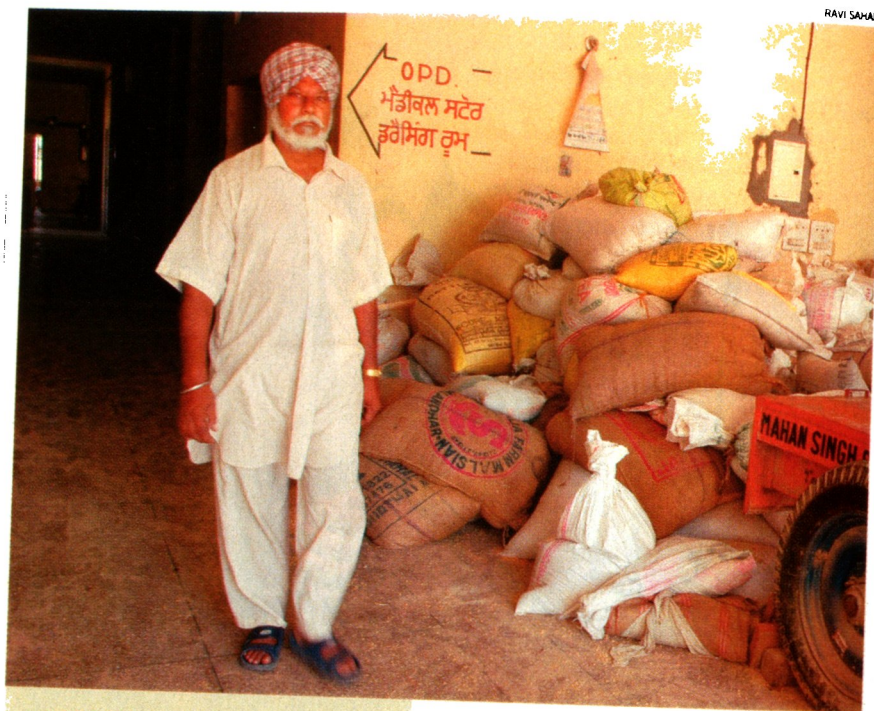


UK resident Balbir Singh Sandhu, vice-president of Britain-based Bilga General Hospital Charitable Trust. The trust runs a 70-bed plush hospital in Bilga, a prosperous village about 16 km from Phillaur, where each of the 400-odd households has at least one member settled abroad. Commissioned in April 2005 by public-spirited natives in Britain who pooled Rs 20 crore, the hospital can rival the best private healthcare institutions in the UK. It has, however, been mired in administrative anomalies and its existence is in doldrums due to lack of doctors. "Despite offering highly-subsidised medical care, the hospital is yet to win the public's trust and utilise its infrastructure," says its Ukraine-educated, young medical superintendent B.D.S. Kahlon. Sandhu, however, has a different take: "We are still struggling to garner the support of local stakeholders to run the institution." The trust has put on hold its £51,000 (Rs 42,00,000)-plan for more medical equipment. In fact, the hospitals which have not been able to attract local stakeholders are in deeper trouble. Exemplifying this malaise is the 52-bed hospital at Gura village near Phillaur, which was built by Guru Nanak Niskam Sewak Jatha, a Birmingham-based Sikh body, six years ago, but has since been shut and is now surrounded by waist-high weeds. The Jatha failed to rope in the villagers, who scoff at the Rs 2-crore project as a "sheer waste".

Moreover, engaging the natives to run a hospital has proved an uphill task given the stakes involved and the local factionalism. As in the case of Guru Nanak Dev Charitable Hospital at Pasla. The Rs 3-crore, 50-bed hospital was built on one-and-a-half acres of land donated by an NRI. It was solely funded by London-based first generation immigrants from the village, who wanted to build an ultra-modern heart-care centre. But the hospital has not been able to break even since it was commissioned in 2003, incurring a loss of Rs 50,000 a month. "We can't run it by remote control," rues its London-based trust chairman Sarwan Singh Ubhi.

Barely 2 km from Pasla, another NRI-funded venture, the 50-bed Bhai Sadhu Charitable Hospital, costing



▲ **HOSPITAL:** Sant Baba Kahan Dass Charitable Hospital **Started by** A trust floated by villagers with overseas funds in 1983. Completed in 2003.

LOCATION: KALA SANGHAN, KAPURTHALA

MONEY SPENT: Rs 4 crore

STATUS: Yet to be commissioned

WHO'S TO BLAME: Row over management

Rs 1.5 crore, is gasping for breath. It is plagued by a dearth of doctors and now runs a skeletal OPD. "The hospital is sick for want of surgeons," says part-time doctor Varinder Sharma.

Having spent a fortune on building huge infrastructure, most NRI-initiated hospitals are finding it hard to keep the flow of donations to meet the running costs. The pattern of donations shows that while the initial response is overwhelming, the flow turns into a trickle when it comes to meeting the operational costs. Also, much of the donations by NRIs are aimed at getting tax exemptions in their home country.

Consider the Jasbir Singh Memorial Hospital, built in Vajidpur village in Nawanshahar district, by a US-based society which has invested Rs 2 crore but is yet to get a doctor to run the hospital that promises 24-hour emergency services. "There is no money to hire doctors," says administrator Kirpal Singh, adding, "Since its start in 2005, the hospital has been incurring a loss of Rs 70,000 a month, eating into

the capital money."

But despite the evident sickness of NRI-run hospitals, the trend continues unabated. In Dadiyal, a village on the Chandigarh-Hoshiarpur road, work was started on a five-storey hospital on a two-acre piece of land in 1999. Funded by the Bhagat Hari Singh Charitable Trust floated by the UK-based Sikh sect, the Rs 15-crore hospital promised to be a 128-bed facility, the largest in Hoshiarpur district. However, having already spent Rs 4 crore, the ambitious plans have run into a financial crunch. "We desperately need at least £1 lakh (Rs 82,00,000) to make the first floor functional," says supervisor Amritpal Singh. With the sect head passing away last year, there is a question mark on the possibility of future donations by NRIs.

In a state where 70 per cent of the government dispensaries and primary health centres don't have proper buildings, NRI-financed charitable hospitals are strikingly unutilised or underutilised. "Instead of dumping their dollars on building a parallel health-care infrastructure, NRIs should bolster the government health network," says state Health Minister Luxmi Kanta Chawla. Clearly, the expatriates need to set their priorities right. Otherwise, their donations spurred by nostalgia will only remain what they have turned out to be—wasted charity. ■